

Beyond Verification:

Exploring Resident Attitudes and Behaviors in Code-Status Discussions



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Verifications

Confirming and documenting previously established goals of care

Explorations

Shared decision-making to help the patient make a new or revised decision in light of the clinical situation

Introduction

- Code-status discussions are critical components of ICU admissions and are frequently led by internal medicine residents.
- Discussions are meant to ensure goal-concordant care, but patients often receive uniform, order-focused, and scripted conversations that risk misunderstanding and may not reflect individual goals.
- Discussions can be divided into code-status **verifications** and code-status **explorations**.
- Guidelines recommend personalized discussions, but little is known about what drives residents to conduct code-status verifications versus code-status explorations.

Methods

- Distributed an anonymous electronic survey to all internal medicine and medicine-pediatrics residents at a large urban academic medical center.
- Included five-point Likert-scale and open-ended items assessing the frequency with which residents conduct code-status verifications versus explorations, perceived comfort and preparedness for each, and perceived barriers.
- Summarized answers descriptively as proportions and trends in responses by postgraduate year (PGY) were evaluated by assigning ordinal values to Likert-scale responses and performing T-tests.

Results

28.6% vs. 87.0%

Interns vs. seniors confident conducting explorations (p=0.0001)

94.1%

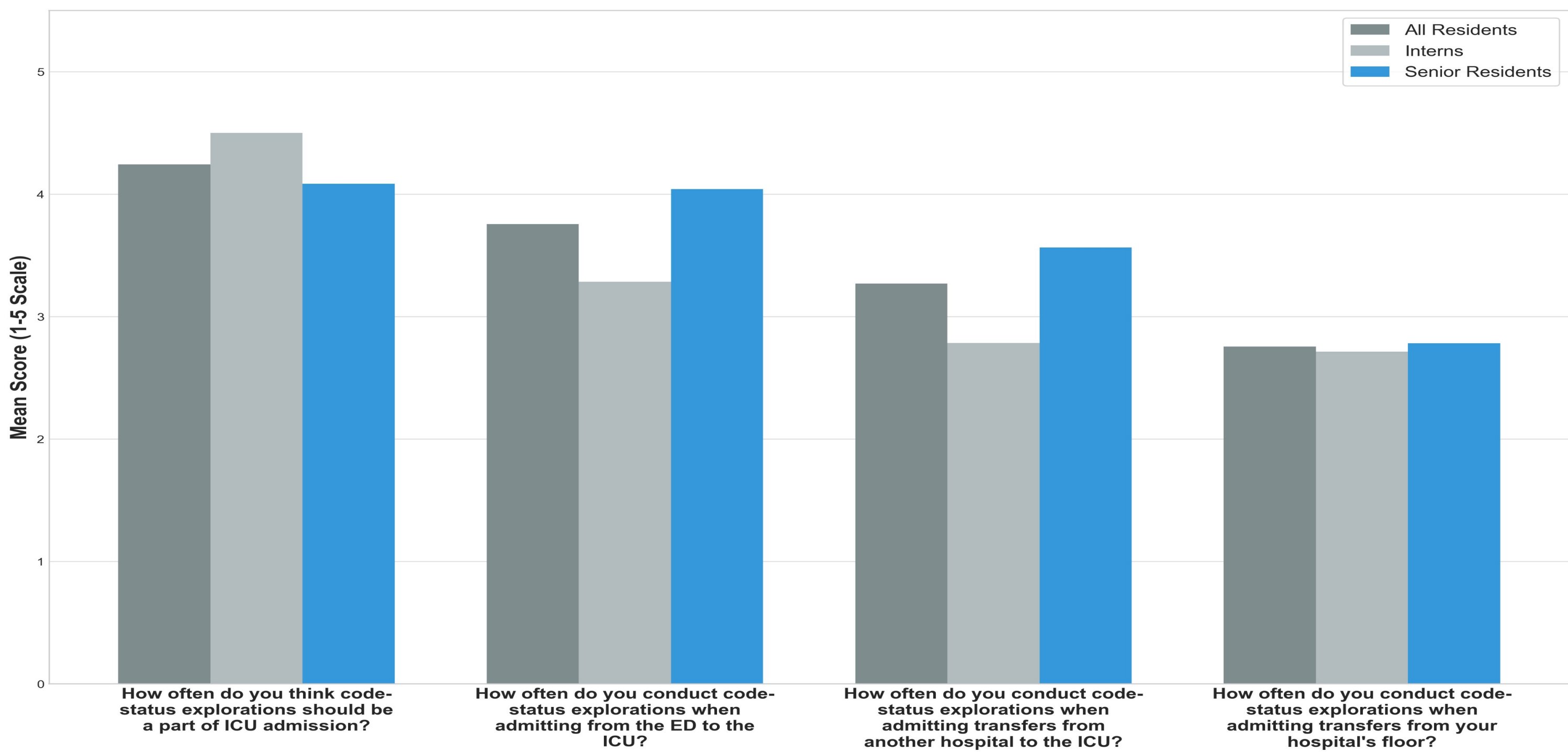
Cite time as a barrier to conducting explorations

EXPLORATIONS BY SOURCE

62% Emergency dept.
49% Outside hospital
27% In-hospital transfer

- 37 residents responded (28% response rate).
- 81.0% believed explorations should "often" or "always" be part of ICU admissions, but fewer reported doing so in practice , especially patients transfers (Figure 1).
- Senior residents (PGY2–4) were numerically more likely than interns (PGY1) to report conducting explorations across all settings.

Figure 1



Senior residents report conducting code-status explorations more often than interns across all admission sources.

- 91.9% reported limitations in patient insight or understanding at least occasionally affected their explorations.

THEMATIC ANALYSIS

Time was the most common barrier to effective code-status explorations identified in residents' open-ended responses.

Discussion

- Residents endorse exploration in principle but conduct it inconsistently, suggesting the gap is structural rather than attitudinal: time pressure and reliance on prior orders push the default toward verification.
- 38.3% reported at least occasionally exploring because they felt it was required rather than clinically indicated. Some exploration may therefore be performative compliance rather than shared decision-making, which is a problem distinct from under-use.
- Intern and senior gaps appear to have different drivers: interns by limited confidence and experience, seniors by patient- and family-level factors such as prognostic uncertainty and readiness. A single training format is unlikely to address both.
- Prior literature has largely treated code-status conversations as a single activity. Residents here draw their own distinction between verification and exploration and apply each differently by context, supporting curricula and decision-support tools that separate the two.

Future Directions

- Test stage-specific interventions: skills-based training for interns; prognostic-communication and workflow tools for senior residents.
- Pilot triage prompts and patient-facing materials to reduce time-driven default to verification.

Conclusions

Residents distinguish exploration from verification but engage in exploration unevenly across context, training level, and time pressure. Targeted curricula in communication, prognostication, and workflow optimization can help ensure code-status discussions remain patient-centered rather than reduced to checklist items.